

RAVENSWOOD MEDICAL GROUP

Endocrinology and Diabetes Care

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OFFICE VISIT SUMMARY

Patient Name: Aaron Nikolaos Garcia
Date of Birth: December 2, 1967
Age: 58

MRN: RMG-004817

Insurance: BCBS of Florida, Plan BCB-FL-GRP-88472931
Visit Date: February 11, 2026
Visit Type: Follow-up, Established Patient (Diabetes Management)
Provider: Anand Patel, MD, FACE
Board Certified, Endocrinology, Diabetes, and Metabolism

REASON FOR VISIT

Routine quarterly diabetes follow-up. Patient presents for A1C review, medication check, and discussion of glucose monitoring options. No acute complaints today.

VITAL SIGNS

Blood Pressure: 141/88 mmHg (right arm, seated)
Heart Rate: 72 bpm (regular)
Temperature: 98.4 F
Weight: 218 lbs
BMI: 31.2 (obese class I)

HISTORY OF PRESENT ILLNESS

Mr. Garcia is a 58-year-old Greek-American male with Type 2 diabetes diagnosed in 2018, currently managed with Metformin and Glipizide. He reports general compliance with his twice-daily Metformin regimen. He checks fasting glucose at home three times weekly and reports readings in the 130 to 150 mg/dL range, with one elevated reading of 178 mg/dL following holiday gatherings in late December 2025 involving shellfish-heavy meals. He denies symptomatic hypoglycemia. He acknowledges that carbohydrate intake is higher during the October-to-April restaurant season. He is on his feet 8 to 10 hours per day, which he counts as his primary physical activity. He reports the January 2026 gout flare (right big toe) has fully resolved. He is currently taking Allopurinol 300mg daily and Amlodipine 10mg daily for hypertension, managed by his PCP Dr. Robert Chen.

LAB RESULTS (Drawn January 28, 2026 at Quest Diagnostics, Tarpon Springs)

HbA1c: 7.2% (Reference: under 7.0 for controlled diabetes)

Fasting Glucose: 138 mg/dL (Reference: 70-99 mg/dL)

Creatinine: 0.94 mg/dL (Reference: 0.74-1.35)

eGFR: 82 mL/min/1.73m² (Normal)

Urine Albumin/Creatinine Ratio: 18 mg/g (Reference: under 30 - normal)

Total Cholesterol: 214 mg/dL (Borderline high)

LDL Cholesterol: 138 mg/dL (Elevated; discussed with PCP)

HDL Cholesterol: 48 mg/dL (Low normal)

Triglycerides: 168 mg/dL (Borderline high)

Uric Acid: 5.8 mg/dL (Reference: 3.5-7.2)

ASSESSMENT AND PLAN

1. Type 2 Diabetes Mellitus - Suboptimally Controlled

A1C of 7.2% remains above the target of 7.0% set at last visit. Improvement is modest from 7.4% in November 2025. Primary drivers appear to be seasonal carbohydrate load and restaurant schedule variability.

Plan:

- Continue Metformin 1000mg twice daily (morning with breakfast, evening with dinner).
- Continue Glipizide 5mg once daily with breakfast.
- Increase frequency of home glucose checks to daily during high-season months (October through April). Log and bring readings to next appointment.
- Discussed Freestyle Libre continuous glucose monitor (CGM). Patient open to the concept but would like more time before committing to wearing a sensor on the forearm during restaurant service. Will revisit at next visit.
- Dietary counseling reinforced: reduce refined carbohydrates (rice, pita, pastries), increase fiber, limit portion size of starchy sides. Referral to registered dietitian offered; patient declined at this time.

2. Cardiovascular Risk

LDL of 138 mg/dL is elevated. PCP (Dr. Robert Chen) has discussed Atorvastatin with patient. Patient is pursuing a diet-first approach per Dr. Chen's documentation. Will monitor and coordinate with PCP. I concur with allowing a trial period through next physical in February 2026.

3. Kidney Function

eGFR 82 and UACR 18 - both within acceptable range. No dose adjustment required for Metformin. Annual monitoring appropriate.

NEXT APPOINTMENT

Lab draw: October 27, 2026 (Quest Diagnostics, Tarpon Springs)

Follow-up visit: October 2026 (patient to call office in mid-September to schedule)

Note: Lab results from the October 27 draw will be reviewed at this visit.

MEDICATIONS (Active as of this visit)

1. Metformin HCl 1000mg - Oral tablet - Twice daily (AM and PM with meals)
2. Glipizide 5mg - Oral tablet - Once daily with breakfast
3. Amlodipine 10mg - Managed by PCP
4. Allopurinol 300mg - Managed by Rheumatologist
5. Ibuprofen 400-600mg PRN - For shoulder, as needed (per Orthopedist)
6. Colchicine 0.6mg PRN - For gout flare, as needed (per Rheumatologist)

No allergies on file.

Provider Signature: Anand Patel, MD, FACE Date: February 11, 2026

NPI: 1487302614

This document is a clinical summary for patient reference. It does not replace the medical record. Questions should be directed to our office at (727) 541-8800.